PRODUCT OVERVIEW

ZYPREXA

olanzapine · coated tablets

Olanzapine for schizophrenia and bipolar mania

ZYPREXA is indicated for the treatment of schizophrenia and of moderate to severe manic episodes. In patients whose manic episode has responded to olanzapine, it is also indicated for preventing recurrence in bipolar disorder.[1]

Approved indications | Dosing | Safety information | Clinical evidence



> 90%	5–20 mg

5-HT2 receptor occupancy at every dose from 5 to 20 mg/day[12]

Mechanism of action

once-daily dose range, individualised[1]

Dosing

> 2,900

patients in the controlled schizophrenia trials[1]

Clinical evidence

48.6%

vs 24.2%

responders at 3 weeks in manic episodes, vs placebo[11]

Clinical evidence

Approved indications

For adults. Comprehensive clinical background for both conditions is available in Disease background.

01

Schizophrenia

Treatment of schizophrenia. Effective in maintaining clinical improvement during continuation therapy in patients who have shown an initial treatment response.[1]

Disease background

02

Moderate to severe manic episodes

As monotherapy or in combination therapy.[1]

Clinical evidence

03

Prevention of recurrence in bipolar disorder

In patients whose manic episode has responded to olanzapine treatment.[1]

Clinical evidence

Not an approved indication: bipolar depression. This site covers ZYPREXA coated tablets; the injectable formulations have separate prescribing information.[2,3]

Mechanism of action

Receptor binding profile

Olanzapine is an antipsychotic, antimanic and mood-stabilising agent with affinity for several receptor families. It has greater affinity for serotonin 5-HT2 than dopamine D2 receptors.[1]

Across the dose range of 5–20 mg/day, 5-HT2 occupancy was above 90%, while D2 occupancy rose with dose to a mean of 76% at 20 mg/day.[12]

Receptor profile and occupancy →

D1–5	5-HT	H1	M1–5	α1
Dopamine	Serotonin 2A/2C, 3, 6	Histamine	Muscarinic	Adrenergic

Receptor families at which olanzapine showed affinity (Ki < 100 nM) in preclinical studies. Order does not indicate relative affinity.[1]

Dosing at a glance

Administration & dose range

Administered once daily with or without food. The licensed daily dose range is **5–20 mg/day**, individualised according to clinical status.[1]

Dose escalations, when indicated, should occur at intervals of not less than 24 hours to ensure proper clinical assessment.[1]

Consider a **5 mg/day** starting dose in elderly patients (≥ 65 years) or those with renal or moderate hepatic impairment.[1]

Dose range and special populations →

Recommended starting doses

Indication	Starting dose
Schizophrenia	10 mg
Manic episode, monotherapy	15 mg
Manic episode, combination therapy	10 mg
Prevention of recurrence in bipolar disorder	10 mg

Important safety information

Contraindications

Hypersensitivity to olanzapine or any excipient; known risk of narrow-angle glaucoma.[1]

Dementia-related psychosis

Not recommended: increased mortality and risk of cerebrovascular accident in elderly patients.[1]

Weight and metabolic changes

Monitor weight, blood glucose and lipids at baseline and regularly during treatment.[1]

Neuroleptic malignant syndrome

If signs occur, all antipsychotic medicines, including olanzapine, must be discontinued.[1]

All warnings and monitoring → **Adverse reactions →**

Clinical evidence at a glance

Schizophrenia

Significantly greater improvement in negative and positive symptoms in 2 of 2 placebo-controlled and 2 of 3 comparator-controlled trials.[1]

Trials →

Bipolar disorder · Manic episodes

Greater reduction in manic symptoms than placebo and divalproex, and as add-on to lithium or valproate.[1]

Trials →

Bipolar disorder · Recurrence prevention

Superior to placebo over 12 months; non-inferior to lithium (recurrence 30.0% vs 38.3%). Not superior as 18-month add-on to lithium or valproate.[1]

Trials →

PRESCRIBING INFORMATION

Consult the prescribing information before prescribing

ZYPREXA is a prescription medicine. Treatment should be initiated and monitored by an appropriately qualified healthcare professional.

View full prescribing information

Next screen: **Disease background** →

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